



**Product Summary for
Critical Protector Life (IV) on AIA Guaranteed Protect Plus (IV)**
Version 2.0

These insurance plans are underwritten by AIA Singapore Private Limited (Reg. No. 201106386R) (“we, our, us, AIA Singapore”).

Critical Protector Life (IV) on AIA Guaranteed Protect Plus (IV) (“**CPL (IV) on GPP (IV)**”) is a supplementary benefit that provides Critical Illness coverage. It comprises 2 components:

- **CPL (IV) on GPP (IV) Accumulator** is a **participating** component which contributes to the experience of the participating fund.
- **CPL (IV) on GPP (IV) Booster** is a **non-participating** component which forms part of the Minimum Critical Illness Benefit, if a Critical Illness claim occurs before its expiry date which is the policy anniversary occurring on or immediately following the Insured’s 65th or 75th birthday (depending on your chosen multiplier cutoff age of 65 or 75). It does not come with any cash surrender value.

This is an accelerated benefit that accelerates the payment of the Insured Amount of AIA Guaranteed Protect Plus (IV) (“**GPP (IV)**”).

Product Benefits

Critical Illness Benefit

If the Insured is diagnosed with any of the covered critical illnesses or undergoes a surgical procedure for a Critical Illness covered in the Critical Protector Life (IV) (“**CPL (IV)**”) supplementary benefit, we will pay as follows:

Claim Event Date	Critical Illness Benefit
Before the Expiry Date of CPL (IV) on GPP (IV) Booster component	The Acceleration Percentage of the higher of: (i) Minimum Critical Illness Benefit; or (ii) Insured Amount of CPL (IV) under the GPP (IV) Accumulator component plus applicable bonuses added to your basic policy, after deducting any amounts owing to us (including any outstanding Deferred Premium Amount if the Premium Pass Option is exercised) under your policy. The Minimum Critical Illness Benefit is equal to the sum of the Insured Amount under the CPL (IV) on GPP (IV) Accumulator component and the Insured Amount under the CPL (IV) on GPP (IV) Booster component as at the issue date of your policy (or otherwise adjusted due to claim or any voluntary reduction in Insured Amount as requested by you).
On or after the Expiry Date of CPL (IV) on GPP (IV) Booster component	The Acceleration Percentage of the Insured Amount of CPL (IV) under the GPP (IV) Accumulator component plus applicable bonuses added to your policy, after deducting any amounts owing to us (including any outstanding Deferred Premium Amount if the Premium Pass Option is exercised) under your policy.

Expiry Date of CPL (IV) on GPP (IV) Booster component refers to the policy anniversary occurring on or immediately following the Insured’s 65th birthday (for multiplier cutoff age 65) or 75th birthday (for multiplier cutoff age 75).

The Acceleration Percentage is the lower of:

- (a) Claim Percentage; or
- (b) Maximum Claim Limit divided by the higher of:
 - (i) Minimum Critical Illness Benefit for CPL (IV); or
 - (ii) Insured Amount under the CPL (IV) on GPP (IV) Accumulator component plus applicable bonuses added to your basic policy.



The Claim Percentage and Maximum Claim Limit is as follows:

	Claim Percentage	Maximum Claim Limit
Angioplasty & Other Invasive Treatments for Coronary Artery	10%	S\$25,000
Critical Illnesses covered under CPL (IV) (except for Angioplasty & Other Invasive Treatments for Coronary Artery)	100%	Not applicable

There shall only be one claim payment on Angioplasty and Other Invasive Treatment for Coronary Artery.

Payment of the Critical Illness Benefit reduces the Insured Amount of your basic policy and CPL (IV) by the Acceleration Percentage of the Insured Amount under CPL (IV) supplementary benefit.

Any benefits subsequently payable under the basic policy and CPL (IV) supplementary benefit and any future premiums will be payable based on the reduced Insured Amount.

Besides other underwriting limits applicable to this plan, this benefit is also subject to the Critical Illness per life limit of S\$3,000,000 (aggregated with other policies or supplementary benefits issued on the same life). For policies issued in other currencies, a conversion rate as determined by the company will apply.

The critical illnesses covered under CPL (IV) supplementary benefit, are:

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| 1. Acquired Brain Damage | 37. Medically Acquired HIV infection |
| 2. Acute Severe Ulcerative Colitis | 38. Medullary Cystic Disease |
| 3. Addison disease or Autoimmune Adrenalitis | 39. Motor Neurone Disease [^] |
| 4. Adrenalectomy for Adrenal Adenoma | 40. Multiple Root of Branchial Plexus Injury |
| 5. Alzheimer's Disease / Severe Dementia [^] | 41. Multiple Sclerosis [^] |
| 6. Angioplasty & Other Invasive Treatment for Coronary Artery [^] | 42. Muscular Dystrophy [^] |
| 7. Benign Brain Tumour [^] | 43. Necrotising Fasciitis |
| 8. Biliary Atresia having undergone Liver Transplantation | 44. Occupationally Acquired Hepatitis B or C |
| 9. Blindness (Irreversible Loss of Sight) [^] | 45. Open-Heart Heart Valve Surgery [^] |
| 10. Brain Surgery | 46. Osteogenesis Imperfecta |
| 11. Chronic Auto-Immune Hepatitis | 47. Other Serious Coronary Artery Disease [^] |
| 12. Chronic Relapsing Pancreatitis | 48. Paralysis (Irreversible Loss of Use of Limbs) [^] |
| 13. Coma [^] | 49. Persistent Severe Juvenile Rheumatoid Arthritis |
| 14. Coronary Artery By-pass Surgery [^] | 50. Persistent Vegetative State (Apallic Syndrome) [^] |
| 15. Creutzfeldt-Jakob Disease | 51. Pheochromocytoma |
| 16. Deafness (Irreversible Loss of Hearing) [^] | 52. Poliomyelitis [^] |
| 17. Ebola | 53. Primary Pulmonary Hypertension [^] |
| 18. Elephantiasis | 54. Progressive Scleroderma [^] |
| 19. End Stage Kidney Failure [^] | 55. Progressive Supranuclear Palsy |
| 20. End Stage Liver Failure [^] | 56. Rabies |
| 21. End Stage Lung Disease [^] | 57. Resection of the whole small intestine (duodenum, jejunum and ileum) |
| 22. Fulminant Hepatitis [^] | 58. Severe Bacterial Meningitis [^] |
| 23. Generalized Tetanus | 59. Severe Cardiomyopathy |
| 24. Heart Attack of Specified Severity [^] | 60. Severe Crohn's Disease |
| 25. HIV Due to Blood Transfusion and Occupationally Acquired HIV [^] | 61. Severe Eisenmenger's Syndrome |
| 26. Idiopathic Parkinson's Disease [^] | 62. Severe Encephalitis [^] |
| 27. Infective Endocarditis | 63. Severe Haemophilia |
| 28. Insulin Dependent Diabetes Mellitus | 64. Severe Myasthenia Gravis |
| 29. Irreversible Aplastic Anaemia [^] | 65. Severe Pulmonary Fibrosis |
| 30. Irreversible Loss of Speech [^] | 66. Stroke with Permanent Neurological Deficit [^] |
| 31. Juvenile Huntington Disease | 67. Surgery for Idiopathic Scoliosis |
| 32. Loss of Independent Existence [^] | 68. Surgery to Aorta [^] |
| 33. Major Burns [^] | 69. Systemic Lupus Erythematosus with Lupus Nephritis [^] |
| | 70. Terminal Illness [^] |



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| 34. Major Cancer^ | 71. Tuberculosis Meningitis |
| 35. Major Head Trauma^ | 72. Type 1 Juvenile Spinal Muscular Atrophy |
| 36. Major Organ / Bone Marrow Transplantation^ | 73. Wilson's Disease |

^ The Life Insurance Association Singapore (LIA) has standard Definitions for 37 severe-stage Critical Illnesses (Version 2024). These Critical Illnesses fall under Version 2024. You may refer to www.lia.org.sg for the standard Definitions (Version 2024). For Critical Illnesses that do not fall under Version 2024, the definitions are determined by the insurance company.

The definitions of the above critical illnesses are contained in Appendix A.

Key Product Provisions

The following are some key provisions found in the supplementary agreements of these supplementary benefits. This is only a brief summary and you are advised to refer to the actual terms and conditions in the supplementary agreement. Please consult your AIA Financial Services Consultant or Insurance Representative should you require further explanation.

1) Premium

Premium rates for CPL (IV) are not guaranteed. We may revise the premium rates but we will not do so on an individual basis. The premium rates may be adjusted based on future experience and any changes or amendments to the laws and regulations of Singapore (including but not limited to tax laws and regulations).

2) Waiting Period

We will not pay any benefits for Heart Attack of Specified Severity, Major Cancer, Coronary Artery By-pass Surgery, Angioplasty & Other Invasive Treatment for Coronary Artery or Other Serious Coronary Artery Disease if the date of diagnosis of the Heart Attack of Specified Severity, Major Cancer, Other Serious Coronary Artery Disease or the date of diagnosis of any conditions leading to performance of Coronary Artery By-pass Surgery or Angioplasty & Other Invasive Treatment for Coronary Artery to the Insured was made within 90 days from the later of:

- (a) the issue date of your basic policy or these supplementary benefits, whichever is later; or
- (b) the reinstatement date of your basic policy or these supplementary benefits, whichever is later.

3) General Exclusions

There are certain conditions under which no benefits will be payable. These are stated as exclusions in the CPL (IV) supplementary agreement. The exclusions for CPL (IV) supplementary benefit include, but are not limited, to the following conditions. You are advised to read the CPL (IV) supplementary agreement for the full list of exclusions.

- (a) illnesses or surgical procedures other than a diagnosis of a Critical Illness, or surgical procedure performed for a Critical Illness in the CPL (IV) supplementary benefit;
- (b) pre-existing illnesses, diseases, impairments or conditions from which the Insured was suffering prior to the issue date or reinstatement date of CPL (IV) supplementary benefit, whichever is later, unless a declaration was made in the application for, or reinstatement of, these supplementary benefits and such application was specifically accepted by us;
- (c) where the diagnosis of Fulminant Hepatitis or Major Cancer of the Insured was, in our opinion, directly or indirectly due to an Acquired Immunodeficiency Syndrome (AIDS) or infection by any Human Immunodeficiency Virus (HIV); or
- (d) deliberate acts that endanger oneself, whether sane or insane, including any of the following:
 - (i) while in violation or attempted violation of the law or resistance to arrest; or
 - (ii) suicide or attempted suicide, intentional self-injury or exposure to exceptional danger (except in an attempt to save human life).



4) Termination

CPL (IV) supplementary benefit shall automatically terminate on the earliest occurrence of the following:

- (a) if any premium for CPL (IV) supplementary benefit remains unpaid at the end of the grace period (except if Premium Pass Option is exercised) and there is insufficient guaranteed cash value under GPP (IV) Accumulator component to grant an automatic policy loan to pay the outstanding premium;
- (b) if your basic policy is terminated;
- (c) if your basic policy is converted to reduced paid-up insurance or extended term insurance;
- (d) upon the diagnosis of a Critical Illness or undergoing of surgical procedure for a Critical Illness covered in the CPL (IV) supplementary benefit (except for Angioplasty & Other Invasive Treatment for Coronary Artery); or
- (e) our receipt of your written request to terminate this supplementary benefit.

In addition, the coverage for CPL (IV) on GPP (IV) Booster component shall automatically terminate on its expiry date, which is the policy anniversary occurring on or immediately following the Insured's 65th birthday (for multiplier cutoff age 65) or 75th birthday (for multiplier cutoff age 75).

The payment of any premium subsequent to termination of this supplementary benefit should not create any liability for us.

Important Notes

All insurance applications are subject to our underwriting and acceptance. Submission of an application and payment of premium does not constitute and should not be construed as acceptance by us. We reserve the right to withdraw the plans or reject applications, at any time or for any reason without notice.

This product summary does not form a part of any contract of insurance. It is intended only to be a simplified description of the product features applicable to these supplementary benefits and is not exhaustive. The contents of this product summary may vary from the terms of cover eventually issued. Please refer to the actual supplementary agreements for all terms and conditions, including exclusions whereby the benefits under your policy may not be paid out. You are advised to read the supplementary agreements. For the avoidance of doubt, only the terms and conditions as set out in the supplementary agreements will bind the parties.

Buying a life insurance policy can be a long-term commitment. You should consider carefully before terminating the policy or switching to a new one as there may be disadvantages in doing so. The new policy may cost more or have fewer benefits at the same cost. Buying health insurance products that are not suitable for you may impact your ability to finance your future healthcare needs.



Appendix A
Definition of the Critical Illnesses applicable to:
Critical Protector Life (IV) on AIA Guaranteed Protect Plus (IV)

Critical Illnesses and Definition	
1	Acquired Brain Damage Acquired Brain Damage refers to a condition where all of the following conditions must be met: • the Insured has attained the age of four (4) years old or above; • brain imaging studies and neuro-psychological testing appropriate to the Insured's age have confirmed the presence of moderate to severe brain damage; and • the development of the Insured is delayed by the equivalent of at least two (2) years and there is a need for special childcare and special schooling as confirmed by a Specialist in the relevant field. Brain damage as a result of congenital causes is excluded. Coverage will end on the policy anniversary occurring on or immediately following the Insured's 21st birthday.
2	Acute Severe Ulcerative Colitis The Acute Severe Ulcerative Colitis is defined as emergency medical condition and requires hospitalization for prompt treatment. The unequivocal Diagnosis of Acute Severe Ulcerative Colitis is finalised by gastroenterologist and being supported by all of following criteria: • Frequency of bloody stools (≥ 6 per day) and • at least one marker of systemic toxicity: pulse rate >90 bpm, temperature $>37.8^{\circ}\text{C}$, haemoglobin <10.5 g/dl and/or an ESR >30 mm/h, • Ulcerative Colitis is uncontrolled by medication and surgery of colectomy is done. • The Crohn's disease is excluded.
3	Addison disease or Autoimmune Adrenalitis Addison disease (or Addison's disease, autoimmune adrenalitis) is adrenocortical insufficiency due to the destruction or dysfunction of the entire adrenal cortex. The unequivocal Diagnosis is confirmed by endocrinologist. The Diagnosis of Addison disease (or Addison's disease, autoimmune adrenalitis) must be supported by all of following: • There is raised of blood ACTH greater than 50 pg/ml • There is evidence of no response of raised aldosterone (serum cortisone) with ACTH test. • There is a need for life long glucocorticoid and mineral corticoid replacement therapy. Only autoimmune cause of primary adrenal insufficiency is included. All other causes of adrenal insufficiency are excluded.
4	Adrenalectomy for Adrenal Adenoma The actual undergoing of Adrenalectomy for treatment of poorly controlled systemic hypertension that was secondary to an aldosterone secreting adrenal adenoma and was uncontrolled by medical therapy. The adrenalectomy would have to be deemed necessary for the management of poorly controlled hypertension by a Specialist.
5	Alzheimer's Disease / Severe Dementia Deterioration or loss of cognitive function as confirmed by clinical evaluation and imaging tests, arising from Alzheimer's disease or irreversible organic disorders, resulting in significant reduction in mental and social functioning requiring the continuous supervision of the Insured. This Diagnosis must be supported by the clinical confirmation of an appropriate consultant and supported by our appointed Physician.



Critical Illnesses and Definition	
	The following are excluded: • Non-organic diseases such as neurosis and psychiatric illnesses; and • Alcohol related brain damage.
6	Angioplasty & Other Invasive Treatment for Coronary Artery The actual undergoing of balloon angioplasty or similar intra arterial catheter procedure to correct a narrowing of minimum 60% stenosis, of one (1) or more major coronary arteries as shown by angiographic evidence. The revascularisation must be considered medically necessary by a consultant cardiologist. Coronary arteries herein refer to left main stem, left anterior descending, circumflex and right coronary artery. Diagnostic angiography is excluded.
7	Benign Brain Tumour Benign brain tumour means a non-malignant tumour located in the cranial vault and limited to the brain, meninges or cranial nerves where all of the following conditions are met: • It has undergone surgical removal or, if inoperable, has caused a Permanent Neurological Deficit; and • Its presence must be confirmed by a neurologist or neurosurgeon and supported by findings on Magnetic Resonance Imaging, Computerised Tomography, or other reliable imaging techniques. The following are excluded: • Cysts; • Abscess; • Angioma; • Granulomas; • Vascular Malformations; • Haematomas; and • Tumours of the pituitary gland, spinal cord and skull base.
8	Biliary Atresia having undergone Liver transplantation Biliary atresia (BA) is a progressive, idiopathic, fibro-obliterative disease of the extra-hepatic biliary tree that presents with biliary obstruction and has undergone liver transplantation or is on a registered liver transplantation waiting list. The Diagnosis should be confirmed by a gastroenterologist with supporting evidence including imaging, laboratory tests and liver biopsy. Biliary atresia due to other disease is excluded.
9	Blindness (Irreversible Loss of Sight) Permanent and irreversible loss of sight in both eyes as a result of illness or accident to the extent that even when tested with the use of visual aids, vision is measured at 6/60 or worse in both eyes using a Snellen eye chart or equivalent test, or visual field of 20 degrees or less in both eyes. The blindness must be confirmed by an ophthalmologist. The blindness must not be correctable by surgical procedures, implants or any other means.
10	Brain Surgery Brain Surgery refers to the actual undergoing of a craniotomy and medically necessary surgery to the brain under general anaesthesia on the recommendation by a qualified Specialist in the relevant field. Brain Surgery as a result of an accident or burr hole surgery solely to remove a blood clot is excluded.
11	Chronic Auto-Immune Hepatitis A chronic necro-inflammatory liver disorder of unknown cause associated with circulating auto-antibodies and a high serum globulin level.



Critical Illnesses and Definition	
	The Diagnosis must be based on all of the following criteria: • hyper-gammaglobulinaemia • the presence of at least one of the following auto-antibodies: - Anti-Nuclear Antibody; - Anti-smooth muscle antibodies; - Anti-actin antibodies; - Anti-LKM-1 antibodies; - Anti- LC1 antibodies; or - Anti-SLA/LP antibodies • Liver Biopsy confirmation of the Diagnosis of auto-immune hepatitis This is only covered if the Insured is treated with Immunosuppressive therapy for six (6) months duration or is documented to be under the care of Specialist in gastroenterology or hepatology for six (6) months duration.
12	Chronic Relapsing Pancreatitis Multiple attacks of pancreatitis resulting in pancreatic dysfunction causing malabsorption needing enzyme replacement therapy. The Diagnosis must be made by a gastroenterologist and supported by appropriate investigation results. Chronic Relapsing Pancreatitis caused by alcohol or drug abuse is excluded.
13	Coma A coma that persists for at least 96 hours. This Diagnosis must be supported by evidence of all of the following: • No response to external stimuli for at least 96 hours; • Life support measures are necessary to sustain life; and • Brain damage resulting in Permanent Neurological Deficit which must be assessed at least 30 days after the onset of the coma. For the above definition, medically induced coma and coma resulting directly from alcohol or drug abuse or self-inflicted injuries are excluded.
14	Coronary Artery By-pass Surgery The actual undergoing of open-chest surgery or Minimally Invasive Direct Coronary Artery Bypass surgery to correct the narrowing or blockage of one (1) or more coronary arteries with bypass grafts. This Diagnosis must be supported by angiographic evidence of significant coronary artery obstruction and the procedure must be considered medically necessary by a consultant cardiologist. Angioplasty and all other intra-arterial, catheter-based techniques, 'keyhole' or laser procedures are excluded.
15	Creutzfeldt-Jakob Disease The occurrence of Creutzfeldt-Jakob Disease or Variant Creutzfeldt-Jakob Disease where there is an associated neurological deficit, which is solely responsible for a permanent inability to perform at least three (3) of the six (6) "Activities of Daily Living". Disease caused by human growth hormone treatment is excluded.
16	Deafness (Irreversible Loss of Hearing) Total and irreversible loss of hearing in both ears as a result of illness or accident. This Diagnosis must be supported by audiometric and sound threshold tests provided and certified by an Ear, Nose and Throat (ENT) Specialist. Total means "the loss of hearing to the extent that the quietest sound that can be heard is 80 decibels or greater across all frequencies". Irreversible means "cannot be reasonably restored to 40 decibels or lower by medical treatment, hearing aid



	Critical Illnesses and Definition
	and/or surgical procedures consistent with the current standard of the medical services available in Singapore after a period of six (6) months from the date of intervention.”
17	Ebola Infection with the Ebola virus where the following conditions are met: • presence of the Ebola virus has been confirmed by laboratory testing; and • there are ongoing complications of the infection persisting beyond 30 days from the onset of symptoms.
18	Elephantiasis The end-stage lesion of filariasis, characterised by massive swelling in the tissues of the body as a result of obstructed circulation in the blood or lymphatic vessels. Unequivocal Diagnosis of elephantiasis must be: • clinically confirmed by a Physician in the appropriate medical specialty; and • supported by laboratory confirmation of microfilariae Lymphedema caused by infection with any other disease(s), trauma, post-operative scarring, congestive heart failure, or congenital lymphatic system abnormalities is excluded.
19	End Stage Kidney Failure Chronic irreversible failure of both kidneys requiring either permanent renal dialysis or kidney transplantation.
20	End Stage Liver Failure End stage liver failure as evidenced by all of the following: • Permanent jaundice; • Ascites; and • Hepatic encephalopathy. Liver disease secondary to alcohol or drug abuse is excluded.
21	End Stage Lung Disease End stage lung disease, causing chronic respiratory failure. This Diagnosis must be supported by evidence of all of the following: • FEV₁ test results which are consistently less than one (1) litre; • Permanent supplementary oxygen therapy for hypoxemia; • Arterial blood gas analyses with partial oxygen pressures of 55mmHg or less (PaO₂ ≤55mmHg); and • Dyspnea at rest. The Diagnosis must be confirmed by a respiratory Physician.
22	Fulminant Hepatitis A submassive to massive necrosis of the liver by the Hepatitis virus, leading precipitously to liver failure. This Diagnosis must be supported by all of the following: • Rapid decreasing of liver size as confirmed by abdominal ultrasound; • Necrosis involving entire lobules, leaving only a collapsed reticular framework; • Rapid deterioration of liver function tests; • Deepening jaundice; and • Hepatic encephalopathy.
23	Generalized Tetanus Tetanus is an illness characterised by an acute onset of hypertonia, painful muscular contractions (including but not limited to the muscles of the jaw and neck) and generalised muscle spasms caused by tetanus toxin that is produced by Clostridium tetani bacterium infection.



Critical Illnesses and Definition	
	The Diagnosis of Generalised Tetanus due to tetanus toxin must be confirmed by a Physician. All the following criteria must be met to qualify for this benefit: • Constant mechanical ventilation is instituted for at least three (3) days as a medically necessary treatment for Generalised Tetanus due to tetanus toxin; and • Tetanus immune Globulin is administered.
24	Heart Attack of Specified Severity Death of heart muscle due to ischaemia, that is evident by at least three (3) of the following criteria proving the occurrence of a new heart attack: • History of typical chest pain; • New characteristic electrocardiographic changes; with the development of any of the following: ST elevation or depression, T wave inversion, pathological Q waves or left bundle branch block; • Elevation of the cardiac biomarkers, inclusive of CKMB above the generally accepted normal laboratory levels or Cardiac Troponin T or I at 0.5ng/ml and above; • Imaging evidence of new loss of viable myocardium or new regional wall motion abnormality. The imaging must be done by cardiologist specified by us. For the above definition, the following are excluded: • Angina; • Heart attack of indeterminate age; and • A rise in cardiac biomarkers or Troponin T or I following an intra-arterial cardiac procedure including, but not limited to, coronary angiography and coronary angioplasty. Explanatory note: 0.5ng/ml = 0.5ug/L = 500pg/ml
25	HIV Due to Blood Transfusion and Occupationally Acquired HIV A) Infection with the Human Immunodeficiency Virus (HIV) through a blood transfusion, provided that all of the following conditions are met: • The blood transfusion was medically necessary or given as part of a medical treatment; • The blood transfusion was received in Singapore after the Issue Date, date of endorsement or Reinstatement Date of your Policy/ Supplementary Agreement (where applicable), whichever is the later; and • The source of the infection is established to be from the Institution that provided the blood transfusion and the Institution is able to trace the origin of the HIV tainted blood. B) Infection with the Human Immunodeficiency Virus (HIV) which resulted from an accident occurring after the Issue Date, date of endorsement or Reinstatement Date of your Policy/ Supplementary Agreement (where applicable), whichever is the later whilst the Insured was carrying out the normal professional duties of his or her occupation in Singapore, provided that all of the following are proven to our satisfaction: • Proof that the accident involved a definite source of the HIV infected fluids; • Proof of sero-conversion from HIV negative to HIV positive occurring during the 180 days after the documented accident. This proof must include a negative HIV antibody test conducted within five (5) days of the accident; and • HIV infection resulting from any other means including sexual activity and the use of intravenous drugs is excluded. This benefit is only payable when the occupation of the Insured is a medical practitioner, housemen, medical student, state registered nurse, medical laboratory technician, dentist (surgeon and nurse) or paramedical worker, working in medical centre or clinic (in Singapore). This benefit will not apply under either section A or B where a cure has become available prior to the infection. "Cure" means any treatment that renders the HIV inactive or non-infectious.



Critical Illnesses and Definition	
26	Idiopathic Parkinson's Disease The unequivocal Diagnosis of idiopathic Parkinson's Disease by a consultant neurologist. This Diagnosis must be supported by all of the following conditions: • The disease cannot be controlled with medication; and • Inability of the Insured to perform (whether aided or unaided) at least three (3) of the six (6) "Activities of Daily Living" for a continuous period of at least six (6) months. For the purpose of this definition, "aided" shall mean with the aid of special equipment, device and/or apparatus and not pertaining to human aid.
27	Infective Endocarditis Inflammation of the inner lining of the heart caused by infectious organisms, where all of the following criteria are met: • Positive result of the blood culture proving presence of the infectious organism(s); • Presence of at least moderate heart valve incompetence (heart valve regurgitant) or moderate heart valve stenosis attributable to Infective Endocarditis; and • The unequivocal Diagnosis and the severity of valvular impairment are confirmed by a consultant cardiologist and supported by echocardiogram or other reliable imaging technique
28	Insulin Dependent Diabetes Mellitus refers to a condition where all of the following diagnostic conditions must be met: • there is an on-going absence of insulin production by the pancreas due to auto-immune disease; • exogenous insulin administration is medically necessary to maintain normal glucose metabolism as Diagnosed by a consultant endocrinologist; and • the condition has been present for at least six (6) months. Coverage will end on the policy anniversary occurring on or immediately following the Insured's 21st birthday.
29	Irreversible Aplastic Anaemia Chronic persistent and irreversible bone marrow failure, confirmed by biopsy, which results in anaemia, neutropenia and thrombocytopenia requiring treatment with at least one (1) of the following: • Blood product transfusion; • Bone marrow stimulating agents; • Immunosuppressive agents; or • Bone marrow or haematopoietic stem cell transplantation. The Diagnosis must be confirmed by a haematologist.
30	Irreversible Loss of Speech Total and irreversible loss of the ability to speak as a result of injury or disease to the vocal cords. The inability to speak must be established for a continuous period of 12 months. This Diagnosis must be supported by medical evidence furnished by an Ear, Nose, Throat (ENT) Specialist. All psychiatric related causes are excluded.
31	Juvenile Huntington Disease Diagnosis of Juvenile Huntington Disease with genetic test is confirmed by a Specialist who is a Pediatrician. There must be evidence of all the following: • Movement disorder due to Juvenile Huntington Disease • Cognitive disorder due to Juvenile Huntington Disease; and • Behaviour disorder due to Juvenile Huntington Disease



Critical Illnesses and Definition	
	Coverage will end on the policy anniversary occurring on or immediately following the Insured's 21 st birthday.
32	Loss of Independent Existence A condition as a result of a disease, illness or injury whereby the Insured is unable to perform (whether aided or unaided) at least three (3) of the six (6) "Activities of Daily Living", for a continuous period of six (6) months. This condition must be confirmed by our appointed Physician. Non-organic diseases such as neurosis and psychiatric illnesses are excluded. For the purpose of this definition, "aided" shall mean with the aid of special equipment, device and/or apparatus and not pertaining to human aid.
33	Major Burns Third degree (full thickness of the skin) burns covering at least 20% of the surface of the Insured's body.
34	Major Cancer A malignant tumour positively Diagnosed with histological confirmation and characterised by the uncontrolled growth of malignant cells with invasion and destruction of normal tissue. The term Major Cancer includes, but is not limited to leukemia, lymphoma and sarcoma. Major Cancer Diagnosed on the basis of finding tumour cells and/or tumour-associated molecules in blood, saliva, faeces, urine or any other bodily fluid in the absence of further definitive and clinically verifiable evidence does not meet the above definition. For the above definition, the following are excluded: - All tumours which are histologically classified as any of the following: - Pre-malignant; - Non-invasive; - Carcinoma-in-situ (Tis) or Ta; - Having borderline malignancy; - Having any degree of malignant potential; - Having suspicious malignancy; - Neoplasm of uncertain or unknown behaviour; or - All grades of dysplasia, squamous intraepithelial lesions (HSIL and LSIL) and intra epithelial neoplasia; - Any non-melanoma skin carcinoma, skin confined primary cutaneous lymphoma and dermatofibrosarcoma protuberans unless there is evidence of metastases to lymph nodes or beyond; - Malignant melanoma that has not caused invasion beyond the epidermis; - All Prostate cancers histologically described as T1N0M0 (TNM Classification) or below; or Prostate cancers of another equivalent or lesser classification; - All Thyroid cancers histologically classified as T1N0M0 (TNM Classification) or below; - All Neuroendocrine tumours histologically classified as T1N0M0 (TNM Classification) or below; and all pituitary neuroendocrine tumours (PitNET) except Metastatic PitNET and Pituitary Carcinoma; - All tumours of the Urinary Bladder histologically classified as T1N0M0 (TNM Classification) or below; - All Gastro-Intestinal Stromal tumours histologically classified as Stage I or IA according to the latest edition of the AJCC Cancer Staging Manual, or below; - Chronic Lymphocytic Leukaemia less than RAI Stage 3; - All bone marrow malignancies which do not require recurrent blood transfusions, chemotherapy, targeted cancer therapies, bone marrow transplant, haematopoietic stem cell transplant or other major interventionist treatment; and - All tumours in the presence of HIV infection.
35	Major Head Trauma Accidental head injury resulting in Permanent Neurological Deficit to be assessed no sooner than six (6) weeks from the date of the accident. This Diagnosis must be confirmed by a consultant neurologist and supported by



Critical Illnesses and Definition	
	relevant findings on Magnetic Resonance Imaging, Computerised Tomography, or other reliable imaging techniques. "Accident" means an event of violent, unexpected, external, involuntary and visible nature which is independent of any other cause and is the sole cause of the head Injury. The following are excluded: • Spinal cord injury; and • Head injury due to any other causes.
36	Major Organ / Bone Marrow Transplantation The receipt of a transplant of: • Human bone marrow using haematopoietic stem cells preceded by total bone marrow ablation; or • One (1) of the following human organs: heart, lung, liver, kidney, pancreas that resulted from irreversible end stage failure of the relevant organ. Other stem cell transplants are excluded.
37	Medically Acquired HIV infection The Insured being infected by Human Immunodeficiency Virus (HIV) provided that: • The infection is due to an operation or a medical or dental procedure after the Issue Date, date of endorsement or Reinstatement Date of your Policy/ Supplementary Agreement (where applicable); and • The institution which provided the operation or the medical or dental procedure admits liability or there is a final court verdict that cannot be appealed indicating such liability; and • The infected Insured is not a haemophiliac. The incident must have been reported to appropriate authorities and have been investigated in accordance with the established procedures. This benefit will not apply in the event that any medical cure is found for AIDS or the effects of the HIV virus or a medical treatment is developed that results in the prevention of the occurrence of AIDS. Infection in any other manner, including infection as a result of sexual activity or recreational intravenous drug use is excluded. The insurer must have open access to all blood samples of the Insured and reserves the right to obtain independent testing of such blood samples.
38	Medullary Cystic Disease Medullary Cystic Disease where the following criteria are met: • the presence in the kidney of multiple cysts in the renal medulla accompanied by the presence of tubular atrophy and interstitial fibrosis; • clinical manifestations of anaemia, polyuria, and progressive deterioration in kidney function; and • the Diagnosis of Medullary Cystic Disease is confirmed by renal biopsy. Isolated or benign kidney cysts are specifically excluded from this benefit.
39	Motor Neurone Disease Motor neurone disease characterised by progressive degeneration of corticospinal tracts and anterior horn cells or bulbar efferent neurones which include spinal muscular atrophy, progressive bulbar palsy, amyotrophic lateral sclerosis and primary lateral sclerosis. This Diagnosis must be confirmed by a neurologist as progressive and resulting in Permanent Neurological Deficit.
40	Multiple Root of Brachial Plexus Injury The complete and permanent loss of use and sensory functions of an upper extremity caused by Injury of 2 or more nerve roots of the brachial plexus through accident or disease.



	Critical Illnesses and Definition
	Complete injury of 2 or more nerve roots should be confirmed by electrodiagnostic study or imaging technique done by physiatrist or consultant neurologist.
41	Multiple Sclerosis The definite Diagnosis of Multiple Sclerosis, and must be supported by all of the following: • Investigations which unequivocally confirm the Diagnosis to be Multiple Sclerosis; and • Multiple neurological deficits which occurred over a continuous period of at least six (6) months. Other causes of neurological damage such as SLE and HIV are excluded.
42	Muscular Dystrophy The unequivocal Diagnosis of muscular dystrophy must be made by a consultant neurologist. The condition must result in the inability of the Insured to perform (whether aided or unaided) at least three (3) of the six (6) "Activities of Daily Living" for a continuous period of at least six (6) months. For the purpose of this definition, "aided" shall mean with the aid of special equipment, device and/or apparatus and not pertaining to human aid.
43	Necrotising Fasciitis The occurrence of necrotising fasciitis where the following conditions are met: • the usual clinical criteria of necrotising fasciitis are met; • the bacteria identified is a known cause of necrotising fasciitis; and • there is widespread destruction of muscle and other soft tissues that results in a total and permanent loss of function of the affected body part.
44	Occupationally Acquired Hepatitis B or C Infection with the Hepatitis B or C virus which resulted from an accident occurring after the Issue Date, date of endorsement or Reinstatement Date of your Policy/ Supplementary Agreement (where applicable), whichever is the later whilst the Insured was carrying out the normal professional duties of his or her occupation, provided that all of the following are proven to Our satisfaction: • Proof of the accident giving rise to the infection must be reported to us within 30 days of the accident taking place; • Proof that the accident involved a definite source of the hepatitis B or C infected fluids; • There is a need for antiviral therapy as a consequence of proven seroconversion; • Hepatitis B or C infection resulting from any other means including sexual activity and the use of intravenous drugs is excluded. This benefit is only payable when the occupation of the Insured is a Physician, housemen, medical student, state registered nurse, medical laboratory technician, dentist (surgeon and nurse) or paramedical worker, working in medical centre or clinic. We would not be liable if there had been failure to observe any proper defined procedural practice or occupation required vaccination practices.
45	Open-Heart Heart Valve Surgery The actual undergoing of open-heart surgery to replace or repair heart valve abnormalities. The Diagnosis of heart valve abnormality must be supported by cardiac catheterisation or echocardiogram and the procedure must be considered medically necessary by a consultant cardiologist. The open-heart surgery refers to an incision on the heart for the direct visual replacement or repair of the heart valve abnormalities. For the above definition, the following operation or procedures are excluded:



Critical Illnesses and Definition	
	- The operation or procedure performed via endoscopic or keyhole surgery - The operation or procedure performed via catheterisation
46	Osteogenesis Imperfecta This is a genetic disorder characterised by brittle, osteoporotic, easily fractured bones. The Insured must be Diagnosed as a type III Osteogenesis Imperfecta confirmed by the occurrence of all of the following conditions: - the result of physical examination of the Insured by a Specialist in the relevant field that the Insured suffers from growth retardation and hearing impairment; and - the result of X-ray studies reveals multiple fracture of bones and progressive kyphoscoliosis; and - positive result of skin biopsy. The Diagnosis must be confirmed by a Specialist in the relevant field.
47	Other Serious Coronary Artery Disease The narrowing of the lumen of at least one (1) coronary artery by a minimum of 75% and of two (2) others by a minimum of 60%, as proven by invasive coronary angiography, regardless of whether or not any form of coronary artery surgery has been performed. The occurrence of the stenosis of the involved coronary arteries must be detected in a single invasive coronary angiography report performed in one sitting. Diagnosis by Imaging or non-invasive diagnostic procedures such as CT scan or MRI does not meet the confirmatory status required by the definition. Coronary arteries herein refer to left main stem, left anterior descending, circumflex and right coronary artery. The branches of the above coronary arteries are excluded.
48	Paralysis (Irreversible Loss of Use of Limbs) Total and irreversible loss of use of at least two (2) entire limbs due to injury or disease persisting for a period of at least six (6) weeks and with no foreseeable possibility of recovery. This condition must be confirmed by a consultant neurologist. Self-inflicted injuries are excluded.
49	Persistent Severe Juvenile Rheumatoid Arthritis The unequivocal Diagnosis of Rheumatoid Arthritis by a consultant rheumatologist, with widespread joint destruction and major clinical deformity of at least three (3) of the following joints area: - Hands; - Wrists; - Elbows; - Knees; - Hips; - Ankle; - Cervical spine; or - Metatarsophalangeal joints in the feet The symptoms of arthritis must have persisted for at least one (1) year. Coverage will end on the policy anniversary occurring on or immediately following the Insured's 21st birthday.
50	Persistent Vegetative State (Apallic Syndrome) Universal necrosis of the brain cortex with the brainstem intact. This Diagnosis must be definitely confirmed by a consultant neurologist. This condition has to be medically documented for at least one (1) month.
51	Pheochromocytoma Pheochromocytomas are tumors originating in the catecholamine-producing chromaffin cells of the adrenal medulla.



	Critical Illnesses and Definition
	The Diagnosis of Pheochromocytoma must be confirmed by an endocrinologist. There is actual undergoing of surgery to remove the tumour. Neuroendocrine tumour (NET) is specifically excluded.
52	Poliomyelitis The occurrence of Poliomyelitis where the following conditions are met: • Poliovirus is identified as the cause, • Paralysis of the limb muscles or respiratory muscles must be present and persist for at least three (3) months. The Diagnosis must be confirmed by a consultant neurologist or Specialist in the relevant medical field.
53	Primary Pulmonary Hypertension Primary Pulmonary Hypertension with substantial right ventricular enlargement confirmed by investigations including cardiac catheterisation, resulting in permanent physical impairment of at least Class IV of the New York Heart Association (NYHA) Classification of Cardiac Impairment. The NYHA Classification of Cardiac Impairment: Class I No limitation of physical activity. Ordinary physical activity does not cause undue fatigue, dyspnea, or anginal pain. Class II Slight limitation of physical activity. Ordinary physical activity results in symptoms. Class III Marked limitation of physical activity. Comfortable at rest, but less than ordinary activity causes symptoms. Class IV Unable to engage in any physical activity without discomfort. Symptoms may be present even at rest.
54	Progressive Scleroderma A systemic collagen-vascular disease causing progressive diffuse fibrosis in the skin, blood vessels and visceral organs. This Diagnosis must be unequivocally confirmed by a consultant rheumatologist and supported by biopsy or equivalent confirmatory test and serological evidence and the disorder must have reached systemic proportions to involve the heart, lungs or kidneys. The following are excluded: • Localised scleroderma (linear scleroderma or morphea); • Eosinophilic fasciitis; and • CREST syndrome.
55	Progressive Supranuclear Palsy Progressive Supranuclear Palsy occurring independently of all other causes and resulting in a Permanent Neurological Deficit, which is directly responsible for a permanent inability to perform at least three (3) of the six (6) "Activities of Daily Living". The Diagnosis of Progressive Supranuclear Palsy must be confirmed by a Physician who is a consultant neurologist.
56	Rabies An infection by Rabies virus associated with all of these following signs and symptoms of Rabies namely muscle fasciculations, delirium, psychosis, seizures and aphasia. We will not pay for this Infectious Disease Benefit if the Insured undergoes only the prophylactic post exposure vaccination, without having developed the aforementioned symptoms.
57	Resection of the whole small intestine (duodenum, jejunum and ileum) Complete surgical removal of the whole small intestine including the duodenum, jejunum and ileum as a result of illness or an accident of the Insured.



	Critical Illnesses and Definition
	Partial removal of the small intestine is excluded in this benefit.
58	Severe Bacterial Meningitis Bacterial infection resulting in severe inflammation of the membranes of the brain or spinal cord resulting in significant, irreversible and Permanent Neurological Deficit. The neurological deficit must persist for at least six (6) weeks. This Diagnosis must be confirmed by: • The presence of bacterial infection in cerebrospinal fluid by lumbar puncture; and • A consultant neurologist. Bacterial Meningitis in the presence of HIV infection is excluded.
59	Severe Cardiomyopathy The unequivocal Diagnosis of Cardiomyopathy which have resulted in the presence of permanent physical impairments of at least Class IV of the New York Heart Association (NYHA) classification of Cardiac Impairment. The Diagnosis must be confirmed by a consultant cardiologist. Cardiomyopathy that is directly related to alcohol misuse is excluded. The NYHA Classification of Cardiac Impairment: Class I No limitation of physical activity. Ordinary physical activity does not cause undue fatigue, dyspnea, or anginal pain. Class II Slight limitation of physical activity. Ordinary physical activity results in symptoms. Class III Marked limitation of physical activity. Comfortable at rest, but less than ordinary activity causes symptoms. Class IV Unable to engage in any physical activity without discomfort. Symptoms may be present even at rest.
60	Severe Crohn's Disease Crohn disease is an idiopathic, chronic inflammatory process that can affect any part of the gastrointestinal tract from the mouth to the anus. The unequivocal Diagnosis of Crohn's disease is confirmed by gastroenterologist. To be considered as severe, there must be evidence all of the following criteria: • There is evidence of Crohn's disease in histopathology • The Crohn's disease cannot be controlled with medication. • There is at least one (1) bowel segment resection for treatment of complication of Crohn's disease. The inflammation bowel disease other than Crohn's disease is excluded.
61	Severe Eisenmenger's Syndrome Eisenmenger's Syndrome shall mean the occurrence of a reversed shunt as a result of pulmonary hypertension, caused by a heart disorder. All of the following criteria must be met: • The unequivocal Diagnosis of Eisenmenger's Syndrome is confirmed by consultant cardiologist. • There is history of left to right shunt heart disease before the date of Diagnosis of Eisenmenger's Syndrome. The Diagnosis of left to right shunt heart disease must be supported by echocardiogram or other reliable imaging studies.



	Critical Illnesses and Definition
	• There is evidence of reversed shunt (from left -right shunt to right -left shunt) newly occurred on the date of Diagnosis of Eisenmenger's Syndrome. • Eisenmenger Syndrome has developed to the irreversible stage and there is no any operation available to correct the abnormality. • Presence of permanent physical impairment classified as NYHA IV. The NYHA Classification of Cardiac Impairment: Class I No limitation of physical activity. Ordinary physical activity does not cause undue fatigue, dyspnea, or anginal pain. Class II Slight limitation of physical activity. Ordinary physical activity results in symptoms. Class III Marked limitation of physical activity. Comfortable at rest, but less than ordinary activity causes symptoms. Class IV Unable to engage in any physical activity without discomfort. Symptoms may be present even at rest.
62	Severe Encephalitis Severe inflammation of brain substance (cerebral hemisphere, brainstem or cerebellum) and resulting in Permanent Neurological Deficit which must be documented for at least six (6) weeks. This Diagnosis must be certified by a consultant neurologist, and supported by any confirmatory diagnostic tests. Encephalitis caused by HIV infection is excluded.
63	Severe Haemophilia The Insured must be suffering from severe haemophilia A (VIII deficiency) or haemophilia B (IX deficiency) with factor VIII or factor IX activity levels less than one percent (1%). Diagnosis must be confirmed by a qualified haematologist acceptable to the Us. The coagulation-disease other than haemophilia A (VIII deficiency) or haemophilia B (IX deficiency) are excluded.
64	Severe Myasthenia Gravis An acquired autoimmune disorder of neuromuscular transmission leading to fluctuating muscle weakness and fatiguability, where all of the following criteria are met: • Presence of permanent muscle weakness categorised as Class III, IV or V according to the Myasthenia Gravis Foundation of America Clinical Classification below; and • The Diagnosis of Myasthenia Gravis and categorisation are confirmed by a Physician who is a consultant neurologist. Myasthenia Gravis Foundation of America Clinical Classification: Class I Any eye muscle weakness, possible ptosis, no other evidence of muscle weakness elsewhere Class II Eye muscle weakness of any severity, mild weakness of other muscles Class III Eye muscle weakness of any severity, moderate weakness of other muscles Class IV Eye muscle weakness of any severity, severe weakness of other muscles Class V Intubation needed to maintain airway



Critical Illnesses and Definition	
65	Severe Pulmonary Fibrosis Severe and diffuse type of pulmonary fibrosis requiring extensive and permanent oxygen therapy at least eight (8) hours per day. The unequivocal Diagnosis must be confirmed with lung biopsy and by a Specialist in respiratory medicine.
66	Stroke with Permanent Neurological Deficit A cerebrovascular incident including infarction of brain tissue, cerebral and subarachnoid haemorrhage, intracerebral embolism and cerebral thrombosis resulting in Permanent Neurological Deficit. This Diagnosis must be supported by all of the following conditions: - Evidence of permanent clinical neurological deficit confirmed by a neurologist at least six (6) weeks after the event; and - Findings on Magnetic Resonance Imaging, Computerised Tomography, or other reliable imaging techniques consistent with the Diagnosis of a new stroke. The following are excluded: - Transient Ischaemic Attacks; - Brain damage due to an accident or injury, infection, vasculitis, and inflammatory disease; - Vascular disease affecting the eye or optic nerve; - Ischaemic disorders of the vestibular system; and - Secondary haemorrhage within a pre-existing cerebral lesion.
67	Surgery for Idiopathic Scoliosis The unequivocal Diagnosis of idiopathic scoliosis is confirmed by an orthopaedic surgeon. This scoliosis condition means that the spine curvature angle is equal or more than 40 Cobb angle degree. Surgery to correct abnormal spine curvature to its normal shape (as a straight line viewed from the back) is actually performed. The following conditions are excluded: - scoliosis due to injury or other disease - Kyphosis - Lordosis.
68	Surgery to Aorta The actual undergoing of major surgery to repair or correct an aneurysm, narrowing, obstruction or dissection of the aorta through surgical opening of the chest or abdomen. For the purpose of this definition aorta shall mean the thoracic and abdominal aorta but not its branches. Surgery performed using only minimally invasive or intra arterial techniques are excluded.
69	Systemic Lupus Erythematosus with Lupus Nephritis The unequivocal Diagnosis of Systemic Lupus Erythematosus (SLE) based on recognised diagnostic criteria and supported with clinical and laboratory evidence. In respect of your Policy/ Supplementary Agreement (where applicable), systemic lupus erythematosus will be restricted to those forms of systemic lupus erythematosus which involve the kidneys (Class III to Class VI Lupus Nephritis, established by renal biopsy, and in accordance with the RPS/ISN classification system). The final Diagnosis must be confirmed by a Physician specialising in Rheumatology and Immunology. The RPS/ISN classification of lupus nephritis: Class I Minimal mesangial lupus nephritis Class II Mesangial proliferative lupus nephritis Class III Focal lupus nephritis (active and chronic; proliferative and sclerosing)



Critical Illnesses and Definition	
	Class IV Diffuse lupus nephritis (active and chronic; proliferative and sclerosing; segmental and global) Class V Membranous lupus nephritis Class VI Advanced sclerosis lupus nephritis
70	Terminal Illness Terminal illness means “any condition caused by illness or injury, where at the time of claim, despite all reasonable medical treatment, the Insured is expected to live for no more than 12 months.” The Specialist medical practitioner treating the condition must provide supporting evidence of the condition, possible medical treatment, the prognosis after undergoing the possible medical treatment, and certify that the Insured is expected to live for no more than 12 months despite all possible medical intervention. We reserve the right to appoint an independent medical Specialist who is an expert in the condition to confirm the diagnosis and prognosis. Terminal illness in the presence of HIV infection is excluded.
71	Tuberculosis Meningitis Tuberculosis Meningitis refers to meningitis proven to be caused by mycobacterium tuberculosis that causes a Permanent Neurological Deficit that results in either: (a) severe cognitive impairment documented by standard neuro-psychological that results in the need for continuous supervision; or (b) physical impairment that results in a Permanent inability to perform at least one (1) of the six (6) “Activities of Daily Living”. Meningitis occurring in the presence of HIV infection is excluded.
72	Type 1 Juvenile Spinal Muscular Atrophy Degenerative diseases of the anterior horn cells in the spinal cord and motor nuclei of the brainstem characterised by profound proximal muscular weakness and wasting, primarily in the legs, followed by distal muscle involvement. The unequivocal Diagnosis of Type 1 Juvenile Spinal Muscular Atrophy (SMA) is confirmed by Specialist. The damage must result independently of all other causes and directly in the Insured's permanent inability to perform (whether aided or unaided) at least three (3) of the “Activities of Daily Living” (ADLs) for a continuous period of six (6) months. The Diagnosis must be made by a consultant neurologist with appropriate neuromuscular testing such as Electromyogram (EMG). For the purpose of this definition, “aided” shall mean with the aid of special equipment, device and/or apparatus and not pertaining to human aid. Juvenile Spinal Muscular Atrophy (SMA) other than Type 1 is excluded from this benefit. Coverage will end on the policy anniversary occurring on or immediately following the Insured's 21st birthday.
73	Wilson's Disease A potentially fatal disorder of copper toxicity characterised by progressive liver disease and/or neurologic deterioration due to copper deposit. The Diagnosis must be confirmed by a Specialist in the relevant field and the treatment with a chelating agent must be documented for at least six (6) months.